

I. General information

- Protocol title: **MEDIATING ROLE OF GRIT IN THE RELATIONSHIP BETWEEN WORKPLACE ENVIRONMENT AND TURNOVER INTENTION AMONG NURSES**
- Researcher or Principal Investigator (PI): **XXXX**

II. Background and rationale

Nurses are vital human resources in the hospital. Nurses are responsible for most patient-related responsibilities like recording and monitoring patients' vital signs and health, administering medications, educating patients about disease management, and collaborating with the health care team to plan patient care. According to Haddad and Tony-Butler (2021), there are approximately 29 million nurses and midwives worldwide. The huge number of nurses may impact a positive change that affects the competitiveness and efficiency of a hospital; hence, it is vital to have a sufficient number of nurses (Yoon, 2020). However, as the number of nurses who voluntarily leave (turnover intention) the institution continues to rise, a sufficient number of nurses is in jeopardy. Nurse turnover is defined as the proportion of nurses who have to be replaced in a certain period, compared to the typical number of nurses. It is sometimes referred to as the migration of nurses out of the profession. Many factors can be attributed to nurses migrating from developing to developed countries, such as desiring more professional development opportunities and more significant wage compensation. These factors now increase the level of turnover intention. The turnover intention may increase medication error, limit attention during nursing care, or, worst, drain the hospital of its most important resources—nurses. However, nurse turnover intention remains one of the many issues the nursing profession has faced throughout history.

Turnover intention is a rising healthcare issue linked to the costs and challenges that healthcare institutions confront in recruiting and retaining capable nurses. According to Kaddourah et al. (2018), nurse turnover has a detrimental impact on the capacity to satisfy patient demands and deliver high-quality care, resulting in poor patient outcomes such as higher patient mortality, infection rates, accident rates, resulting in greater burdens for other personnel. Nurse turnover may also have a negative impact on a health care organization's monetary line due to separation and recruitment expenses. Furthermore, according to Gebregziabher et al. (2020), turnover intention is influenced by work-related features such as job prospects, observed independence and flexibility at work, support from colleagues, and job success. These are important research areas because they are associated with improved performance, work ethics, employee enthusiasm, and decreased absenteeism, fatigue, and turnover.

The work environment was one of the ranges of factors that led to the trend of Filipino nurses leaving the country to work abroad since 2012 (Labrague et al. 2018a; Labrague et al. 2018b; Labrague et al. 2017; Castro-Palaganas et al. 2017). Many scholars have sought to conceptualize the term workplace environment over the years. Its most basic form refers to the conditions, settings, situations, and circumstances in which people work. On the other hand, according to Falguera et al. (2021), the Nurse Practice Environment comprises the nurses' participation in hospital affairs, the foundation for quality care, nurse manager's ability, staffing, and resource adequacy, organizational support, and collegial nurse-physician relationships that either improve or limit the professional nursing practice. The direct connection between nurse turnover and work environment has been the focus of research throughout the past decade. For

example, a recent systematic analysis found that nurses who worked in a positive work environment had better job satisfaction and lower turnover intentions (Wei, Sewell, Woody, & Rose, 2018).

A positive work environment manifested by supportive management and adequate resources resulted in fewer adverse events and missed care reports (Lake et al., 2019). According to Kretzschmer et al. (2017), nurses are drawn to a workplace environment that allows them to be more involved in decision-making, promote autonomy, and have good relationships with physicians and other healthcare team members. Positive nurse-physician relationships and adequate nursing personnel and hospital resources were found to considerably minimize the rate of falls among patients, medication errors, and an improved patient experience (Swiger et al., 2018). This is in line with other studies that appropriate staffing to provide high-quality nursing care is an essential constituent of hospital work environments. Nurses' workload is reduced when sufficient staffing allows them to interact more with each patient and render excellent professional care. As a result, patient satisfaction may improve. Li et al. (2019) found out that a work environment with enough material and spiritual resources commences a motivating process that may develop the workers' willingness to devote their efforts and talent to the task, leading to organizational attachment and a low turnover intention. In contrast, nurses who worked in an unpleasant work environment reported poor nursing outcomes, increased workplace violence, greater exhaustion, and hostile work attitudes (Zhang et al., 2017). Furthermore, a poor workplace environment may affect one's grit and their decision whether or not to stay in their workplace. It is a common belief that the employee will give out their best if the employer provides their employee a peaceful and conducive working environment.

Age was also found to be a driver of turnover intention in previous studies. In a study conducted by Martin et al. (2021), age was found to be associated with the employee's organizational attachment and turnover intention where the ever-changing demands, expectations for prospects, and values linked to age are likely to influence an employee's work-related attitudes, including their intention to leave work. Aside from the employee's age, the researchers want to investigate the association between job status and turnover intention. To the researchers' knowledge, there are limited and inconsistent results on the association of the employment status to the nurse's turnover intention in various health settings. However, in a study by Kim et al. (2020), the researchers included the type of employment and how it affects the turnover intention of nurses. They found out that nurses' turnover intentions fluctuate depending on the nurses' employment status. Staff nurses had a larger intention to leave than charge nurses. Nurses with rotational shift work had a higher intention to leave than those with full-time employment.

In relation to the type of institution, private hospitals are owned and operated by a single person or group. On the other hand, public hospitals are managed and funded by the government. The owners of private hospitals are responsible for assuring compliance, operating funds, personnel resources, and hospital equipment, while the government runs the public hospitals. Due to the distinction of the parameters, the relationship may differ depending on whether the medical institution is a private or public institution. Mengstie (2020) stated that both public and private hospital employees had high turnover intentions and that the differences were not statistically significant. Although the difference was not significant and extensive, the turnover score of the participants from public hospitals was slightly higher than those from private hospitals. However, Kakemam et al. (2018) indicated that nurses in private hospitals had significantly lower turnover intentions than their public hospitals counterparts.

Another factor that has been linked to turnover intention is the rise in the number of

cases of COVID-19 patients since it enhanced the workload of health care workers (Lucchini et al., 2020). Most nurses treating COVID-19 patients in underdeveloped countries are at high risk of getting COVID-19 due to a lack of personal protective equipment, leading to an increase in the nurses' intention to quit (Menon & Padhy, 2020). The continuous threat of becoming exposed and infected with COVID-19 increases the nurses' level of anxiety and fear; these negative emotions may lead to high turnover intentions (Barnett & Grabowski, 2020; Mo et al., 2020). In the research conducted by Irshad et al. (2020), the perceived threat of COVID-19 was positively related to psychological anxiety and nurses' turnover intention. Moreover, the research conducted by Jung et al. (2020) found out that nurses caring for patients in infectious disease outbreaks have high turnover intentions.

Previous study has primarily focused on the work-related factors that influence turnover intentions and how to improve job satisfaction, but the personality factor has been overlooked. Our research highlights the need of considering the dispositional factors of working attitudes, including the personal trait of grit. On the other hand, Duckworth and Quinn (2009) characterize grit as a persevering and passionate drive about achieving long-term goals. Grit is frequently related to success and is an excellent psychological trait. As well as that, grit is a term used to describe nurses who have the ability and power to consistently work toward the goals, even if there are difficulties, adversity, and slumps. Grit also alludes to a willingness to move in the direction of hardship, putting forth effort and enthusiasm over time despite setbacks. According to Oh et al. (2019), people with high grit have a sense of efficacy that can rise again even when experiencing complex tasks or challenging situations. The importance of grit in the workplace and other occupational outcomes is unusual. In addition, grit has been investigated as a predictor of work outcomes such as teacher effectiveness, retention, and intention to leave in some research (Jeong et al., 2019). Recent studies also showed the mediating role of grit in the teaching profession. In a study by Hwang et al. (2021), they examined the mediating influence of grit in the effects of job satisfaction, child care, teacher job stress, and emotional commitment on the level of turnover intention. The results of the study indicated the existence of a significant link between grit and retention level. More so, the lesser the turnover intention, the higher the grit (McGinley, 2019). However, grit, the highest predictor of career and personal success, and nurses' turnover intention have not been discussed thoroughly in the nursing literature.

The proposed study can help illuminate the reasons for the fast turnover of nurses in the Philippines. Specifically, in nursing practice, the proposed study will provide a model in assisting with limiting the pace of job turnover and further incrementing the employee's work commitment. This will also help to improve, monitor and remove the practices that are currently applied and removed by evidenced-based practices. In nursing education, the results of this study may be used to increase nurses' positive occupational perceptions through education, training, and lectures to retain future nurses in the healthcare setting. Nursing professors may also utilize various methods or interventions that increase the students' resilience, self-efficacy, motivation, and grit that could impact their nursing practice and retention in the nursing profession in the future.

The nursing administration can utilize the results of this study to conduct regular empowerment programs, control job stressors, develop support measures, and improve negative work environment conditions so that nurses can passionately commit to their profession and provide quality care to patients. The outcomes of this study can be used as a reference by other related studies to further nursing science. This research project could also be replicated and modified by expanding the sample, by comparing nurses in primary, secondary, and tertiary healthcare.

The proposed study will be beneficial to increase the nurses' psychological capital and to improve workplace environment conditions in order to reduce turnover. Decrease turnover intentions of nurses could result in greater retention regarding institution knowledge and experienced workforce, improve the patient safety and satisfaction, and provide better medical outcomes. A more stable workforce could reduce community disruptions caused by the constant turnover of nurses. The proposed study will contribute to the development of organizational, institutional, governmental solutions and strategies, and economic sustainability to reduce the turnover rates for hospital nurses. Examples of these include improving orientation and role transition programs. Implementing shared governance initiatives that offer nurses a voice in scheduling, workflows, and hospital policies may also help.

III. Goals and objectives

Within the healthcare industry, turnover intention is among the issues nurses continue to face, wherein the workplace environment plays a significant factor affecting the sudden intention to leave the workforce. With grit acting as a mediating variable, it can be a potential mechanism by which an independent variable, the workplace, can change a dependent variable, the level of turnover intention. As a result, the current research aims to address the following research questions.

Objectives of the Study

1. What is the level of turnover intention, quality of workplace environment, and grit among nurses based on:
 - A. Age;
 - B. Status of employment;
 - C. Type of institution; and
 - D. Exposure to a positive COVID-19 patient for the last six (6) months?
2. What is the degree of relationship between the level of the workplace environment and turnover intention among nurses?
3. What is the degree of mediating effects of grit between the quality of the workplace environment and nurse's turnover intention?
4. What predictive model can be developed on the mediating effects of grit between the quality of the workplace environment and nurse's turnover intention?

IV. Design

Descriptive-correlational research design will be utilized in the proposed study. Correlational research reflects the direction of the relationship, either positive or negative, between two or more variables. This design can provide initial indications or additional support for theories about causal relationship (Bhandari, 2021).

The population of interest in this study is nurses currently working in selected hospitals around Baguio City and La Trinidad, Benguet. This will be conducted at Hospital X, Hospital Y, and Hospital Z. Hospital X is accessible and affiliated with Saint Louis University-School of Nursing (SLU-SON). It is also the best representative for Private Hospital Nurses around Baguio City. The Institution has been operating for decades, and many nurses and events may affect the nurses' response to turnover intention. Furthermore, Hospital Y and Hospital Z are significantly

large enough to represent Public Hospital Nurses in La Trinidad, Benguet and Baguio City, respectively. The nurses in the area meet the criteria needed for the research. All the institutions are also accommodating and open to research since they have already established affiliations. More than one Institution was chosen to obtain more participants to achieve generalizability.

V. Methodology

Stratified Random Sampling will be used to calculate how many people should be sampled from each subgroup based on the relevant characteristics the researchers have identified. The use of random or systematic sampling to select a sample from 3 private and public hospitals in Benguet Province such as Hospital X, Hospital Y and lastly the Hospital Z with a total of 908 nurses that enables to obtain a sample population that best represents the entire population being studied, making sure that each subgroup of interest is represented.

Initially, the researcher will secure ethics clearance from the Saint Louis University- Research Ethics Committee (SLU-REC). Different institutions may have varying ethics committees, protocols will be followed. After securing the ethics clearance, a letter will be sent to respective gatekeepers asking for permission to conduct the study. The gatekeepers in Hospital X are the Chief Nurses (CN). The letter will be addressed to CN and will also request the institutional or personal email address of the prospective respondents. Politely, head nurses will be shown the endorsed letter by the Chief of Hospital to select nurses to participate in the study. Then when approval to float the questionnaire is secured, the researchers will float the questionnaire.

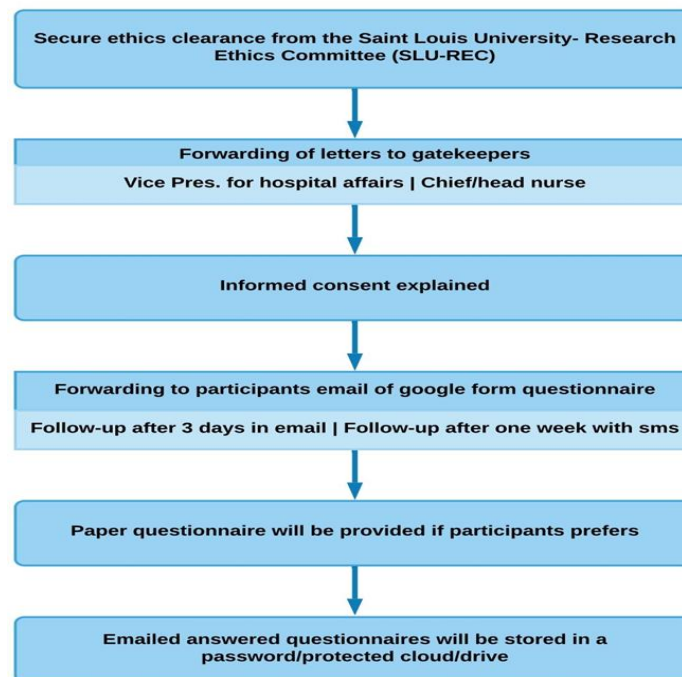


Figure 1. Data gathering procedures

Since email addresses will be requested from the Chief Nurse office. Initially, the informed consent will be explained by the researchers. The researchers plan to obtain the informed consent either virtually (via google meet to reduce human contact) or face-to-face (if necessary). If the circumstances allow face to face, then IATF protocols such as wearing K95

face masks and face shields and observing social distancing will be followed. Then a google form will be sent to the institutional email of the prospective respondents to reduce the risk of getting infected with the COVID-19 virus. Time stamped in the email addresses will be noted in the Annex I (Checklist monitoring of questionnaire administration). If no response is noted, follow-up of the prospective respondents will be done by the research assistant. If the prospective respondents preferred a paper questionnaire, it will be provided. Complete data gathering procedures shown in Figure 2.

The tool to be used in the study will be divided into four sections: demographics, grit Scale, Environmental scale, and Turnover Intention Scale. Specific discussions on the validity and reliability will be subsequently discussed.

In the development and validity of the Short Grit Scale (Duckworth & Quinn, 2009), after excluding two items from each subscale, the resulting eight-item GRIT-S displayed acceptable internal consistency, with alpha ranging from 0.73 to 0.83 across the four samples. The validity for this tool is 0.96 (Duckworth & Quinn, 2009), interpreted as high validity.

In the construct validity and reliability of the Work Environment Assessment Instrument WE-10 (Ahrens, et al. 2020), the Exploratory Factor Analysis (EFA) was used for construct validation, where the Kaiser–Meyer–Olkin (KMO) index was used to verify the suitability of the application, obtaining a KMO value of 0.917 and Bartlett's test ($\chi^2 = 7465,349$, Df = 1225, $p = 0.000$). In terms of reliability, Cronbach's alpha was computed to assess the instrument's internal consistency, with values ranging between 0.953 and 0.956, indicating that all questions were reliable.

In Jacob and Roodt's (2008) study, the Cronbach's alpha coefficient of the survey was 0.913, which was a high and acceptable reliability rating. Martin and Roodt (2008) also reported a high-reliability rating of 0.93 in their research. In analyzing the data results, a higher score is an indication of increased chances of turnover intentions (Taboli, 2015). On the other hand, a Chinese version of TIS was developed by Su (2021), which was used in Mainland China. This Chinese version was created, translated, and then back-translated for cultural adaptation. The C-TIS-6 Confirmatory Factor Analysis result revealed adequate goodness-of-fit with $\chi^2 = 1410.49$, df 149, $p > 0.05$, CFI 0.98, TLI 0.97, RMSEA 0.03, and SRMR 0.05. The factor loadings of the majority of C-TIS-6 items were moderate, ranging from 0.46 to 0.78. Item 3 and 8 had comparatively low factor loadings of 0.22 and 0.24, respectively. Cronbach's alpha coefficient is 0.70, which is regarded as satisfactory in terms of reliability. In the Chinese setting, the C-TIS-6 appears to be a credible measurement scale of turnover intention (Su, 2021).

VI. Safety of research subjects/participants

To minimize the risk of exposure, disease, and transmission of COVID-19, the researchers complied with the COVID-19 Health and Safety Protocols by employing Google Forms as the primary platform for floating questionnaires to reduce face-to-face contact. To ensure maximum safety in instances where there is a need for in-person interaction, such as in coordination with the hospital administrator, complete personal protective equipment (PPE) will be donned by the researchers. In cases of face to face, we will make sure to follow the IATF protocols such as wearing K95 face masks and face shields and observing social distance to minimize the spread of the COVID-19 virus and to make sure all possible contacts will be safe for both researchers and research subjects. The information that will be collected from this research project will be kept private and confidential. No personal identifiable information will be collected such as, but is not limited to: name, address, phone number, government issued numbers and photographs. The researchers will not share any information or linkages with

anyone besides the research team and the panel. The researchers will comply with the Data Privacy Act of 2012 and the Privacy Guidelines of Saint Louis University.

VII. Data management and statistical analysis

Provide information on how the data will be managed, including data handling and coding for computer analysis, monitoring and verification. The statistical methods proposed to be used for the analysis of data should be clearly outlined, including reasons for the sample size selected, power of the study, level of significance to be used, procedures for accounting for any missing or spurious data etc.

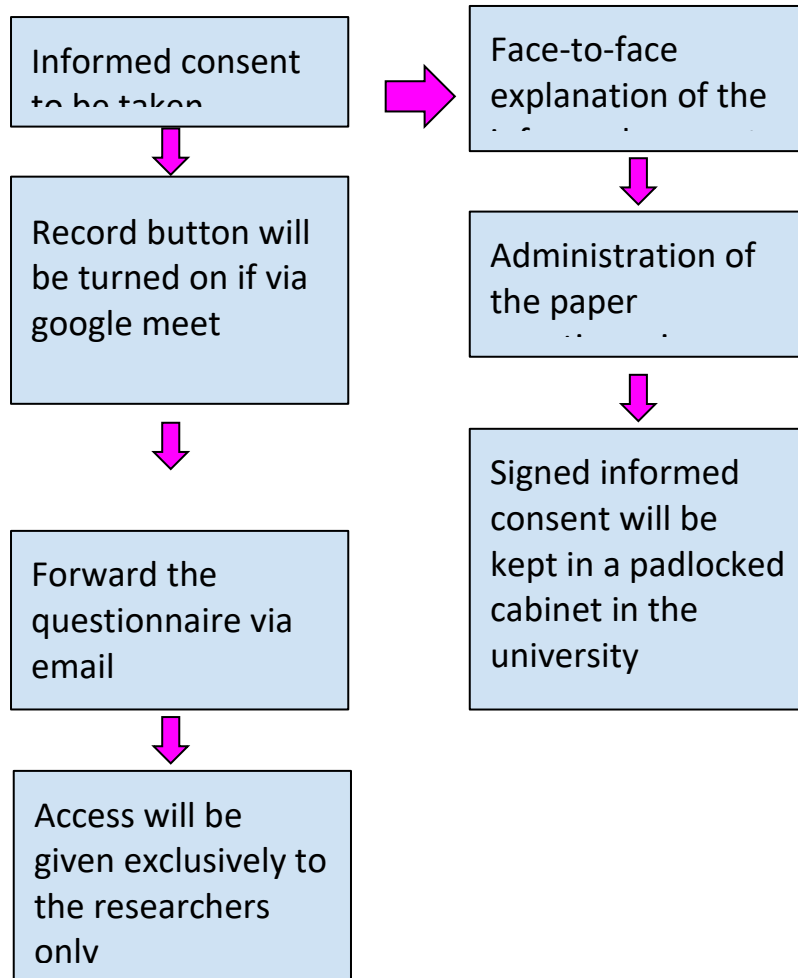


Figure 2. Procedures in obtaining informed consent

In order to ensure privacy of taking the informed consent and questionnaire administration without compromising the IATF protocol, the researchers will obtain informed consent using Google meet. Prior to commencing the process of obtaining informed consent from participants, the record button will be turned on. The objective of activating the recording button is to facilitate documentation. However, to prevent the recorded video from being disclosed, the informed consent recorded video will be protected in a secured and encrypted online cloud (Tresorit Software). Rest assured that access will be restricted only to the

researchers. Then, once the google form questionnaire is completed it will be forwarded via the prospective respondents email addresses.

However, if the informed consent explanation is conducted face-to-face. The researchers will adhere to IATF protocol to ensure the participants' and researchers' safety. After discussing the informed consent process and ensuring that the participants understand it and have no questions about the study, the researchers will begin administering the paper questionnaires. To maintain confidentiality and privacy, the signed informed consent will be stored in a padlocked cabinet at the university.

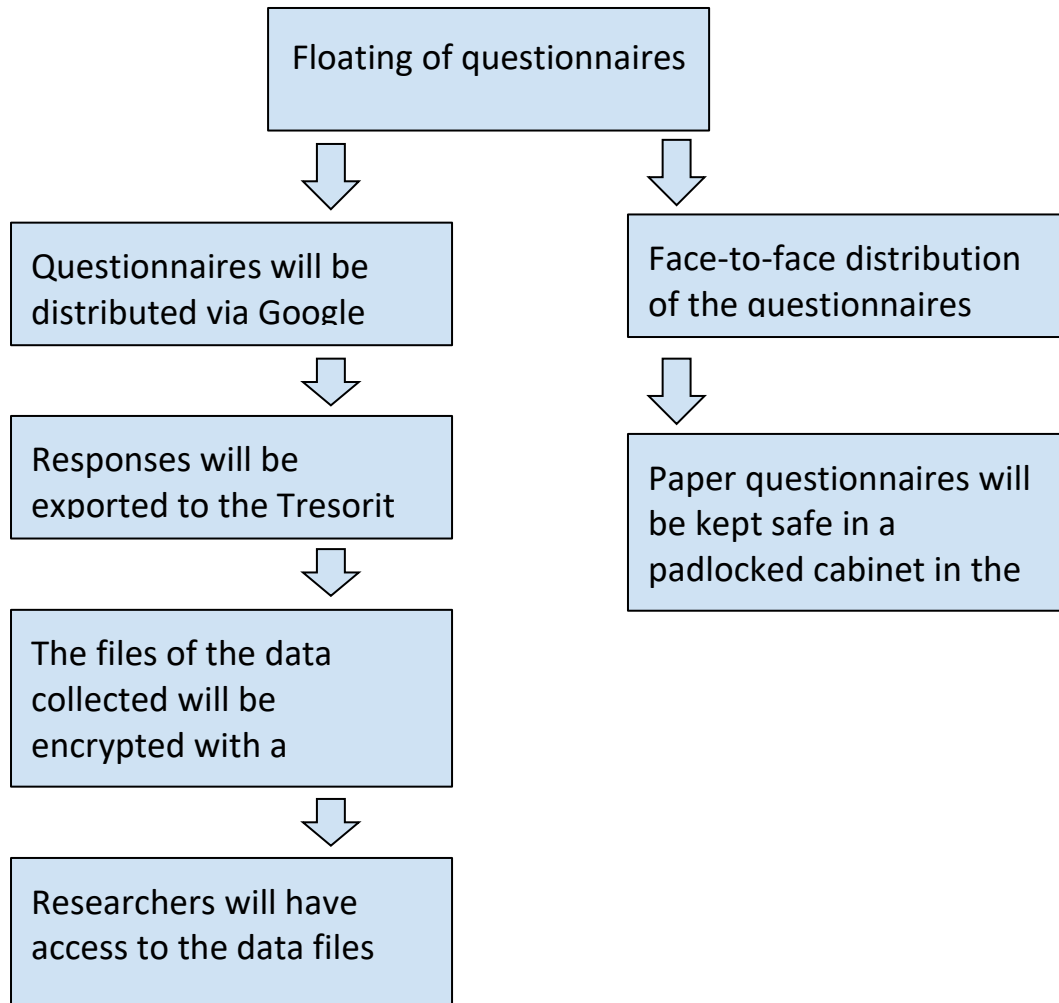


Figure 3. Procedures in floating of questionnaires

To ensure confidentiality of data, the responses collected via google forms will be exported to a security enhanced and data encrypted online cloud, the Tresorit Software and will only be accessible to the researchers. In cases where paper questionnaires are utilized, the responses will be kept in a padlocked cabinet in the university and will only be accessible to the principal investigator and the co-investigators.

VIII. Ethical considerations

Before proceeding to gather data, it is necessary that researchers must strictly follow the ethical considerations – Autonomy, Anonymity, Confidentiality and Right to self-determination. For autonomy, every participant must be fully aware of what the research entails and its implications. Regarding this, people with diminished autonomy such as the elderly, very young children, or mentally disabled persons should be protected because they are considered vulnerable groups. Thus, they cannot make a truly informed decision by themselves and should be reconsidered. They may only be included in research under specific circumstances. Additionally, participants must give their consent to participate without influence, threat or coercion from the researchers and are free to withdraw at any point.

Furthermore, the participants will be ensured that no personal identifiable information will be collected such as, but is not limited to: name, address, phone number, government issued numbers and photographs. The researchers will comply with the Data Privacy Act of 2012 and the Privacy Guidelines of Saint Louis University. The researchers will not share any information or linkages with anyone besides the research team. All data collected will remove any identification of the respondents and change it into a unique ID number. Any information gathered will remain private and confidential. The research team will be the only one who will know the identity of the respondents, and the data collected will not be revealed to anyone. They will be informed what information will be collected and how their identities will be protected. Besides, the consent form includes the nature of data collection and data management procedures to assure the participants' anonymity is protected.

At the same time, confidentiality should be represented when there is an agreement between the researcher and the participant that their individual responses and identities will not be disclosed beyond the research team unless they have agreed otherwise (preferably in writing). The majority of data is now being kept digitally due to technological advancements. As a result, participants' data will be stored on password-protected computers, laptops, file servers, or cloud storage services. Encrypting files also helps preserve the secrecy of digital data that is stored on computer systems or communicated over a network such as the internet.

Finally, researchers must explain the voluntary nature of participation, emphasizing their right to refuse. They must ensure that there is no coercion to potential participants into agreeing to participate in studies. Additionally, giving potential participants extreme rewards for agreeing to participate can also be a form of coercion, because the rewards may provide an enticing-enough incentive that the participant feels they need to participate.

IX. Problems anticipated:

In instances that nurses do not have smartphones or unfamiliar with the platforms use, paper questionnaires will be floated following all the IATF protocols.

X. Dissemination of results and publication policy

The findings of the study will be presented to concerned hospital administrators in the three selected settings. The researchers plan to present the findings in conferences both in the country and abroad. The entire paper is planned to be submitted in a SCOPUS-indexed journals.

XI. Disclosure of potential conflict of interest

No potential conflict of interest in the project.

XII. Financing of project and other support

The project is self-financed by a total number of 12 individuals (1 instructor; 12 students). Expenses: (1) Ethics Review Committee (expedited) - ₱1,500.00, (2) Electricity - ₱2,500.00, (3) Internet Connection - ₱1,500.00, (4) Printing and Paper - ₱500.00. Total Expenses: ₱6,000.00

XIII. Collaboration with other scientists or research institutions

Then proposed study is an undergraduate thesis requirement which does not involve collaboration with other research institutions.

XIV. References

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XV. Appendices (templates can be requested from the University Research and Innovation Center)

A. Protocol Summary Sheet (Form 2.7)

Nurses continue to encounter turnover intentions in the healthcare industry, with the workplace environment playing a critical part in the sudden desire to leave the workforce. The purpose of this descriptive correlational study is to determine the level of turnover intention,

workplace environment quality, and grit among nurses in selected institutions in Benguet, Philippines. The Short Grit Scale (Grit-S), developed by Duckworth et al. (2007), will be used to assess the participants' Grit level. The Work Environment Scale Questionnaire (WES) developed by Insel and Moos (1974) will be used to assess the quality of the workplace environment of participants. The Turnover Intention Scale (TIS) developed by Bothma and Roodt (2013) will measure the participants' turnover intention. The level of turnover intention and work environment will be determined using a weighted mean based on age, employment status, type of institution, and exposure to a positive COVID-19 patient in the previous six months. Measures of position and specific percentiles will be utilized to determine the amount of grit depending on the data provided. The Spearman's Rho coefficient will be used to assess the degree of relationship between the level of the employment environment and the desire to leave. Mediation statistics will be utilized to examine the mediating effects of grit between the quality of the workplace environment and nurses' turnover intention. Participants were 432 registered nurses officially employed in the three selected hospitals and had at least one year of work experience, as determined by stratified random sampling. The proposal will run from August 2021 to May 2022. It is anticipated that it will be completed by the end of the stipulated period, given that the findings of this study would benefit institutions by developing relevant and evidence-based programs to retain the finest nurses and provide better patient outcomes.

B. Informed Consent Form (ICF) and/or Letters of request to concerned offices/communities

This informed consent form is for the employed nurses **in Saint Louis University Hospital of the Sacred Heart, Baguio General Hospital and Medical Center, and Benguet General Hospital**, who we are inviting to participate in our research entitled “Mediating role of grit in the relationship between workplace environment and turnover intention among nurses”

Principal Investigators: XXXX

Name of Organization: Saint Louis University - School of Nursing, Baguio City, Philippines

Name of Proposal: Mediating role of grit in the relationship between workplace environment and turnover intention among nurses.

This Informed Consent Form has two parts:

- **Information Sheet (to share information about the study with you)**
- **Certificate of Consent (for signatures if you choose to participate)**

You will be given a copy of the full Informed Consent Form

Part I: Information Sheet

Introduction

We are BSN III C2 from Saint Louis University under the School of Nursing. We are currently working on the mediating role of grit in the workplace environment and nurse's turnover intention.

You are a vital part in the health care team, especially in this pandemic. You are also responsible for most patient-related responsibilities like administering medications and collaborating with the health care team to plan patient care. However, the nurse's turnover, in general, is getting relatively high in this country and this region, making the country lose its foundation in the healthcare team. We wish to give you information and invite you to be part of this research study. You do not have to decide today whether or not you will participate in the research. Instead, before you decide, you can talk to anyone you feel comfortable with about the study. This consent form may contain words that you do not understand, so please ask for assistance and clarification from the researchers should you have any confusion and queries.

Purpose of the research

We want to know the level of turnover intention, quality of workplace environment, and grit among nurses in selected hospitals in Benguet, Philippines based on age, status of employment, type of institution and absence or presence of an exposure to a positive COVID-19 patient for the last six (6) months. We also want to know the degree of relationship between the level of the workplace environment and turnover intention among nurses in selected hospitals in Benguet and the degree of mediating effects of grit between the quality of the workplace environment and nurse's turnover intention. Furthermore, we want to know what predictive model can be developed on the mediating effects of grit between the quality of the workplace environment and nurse's turnover intention. With this regard, we believe that you can help us achieve all these by participating in our study.

Type of Research Intervention

This research will involve your participation in answering the floated questionnaires that will take about 15-20 minutes to answer.

Participant Selection

You are being invited to take part in this research because we feel that your experience as an employed nurse can contribute much to our understanding and knowledge of the mediating role of grit in the relationship between workplace environment and turnover intention.

Voluntary Participation

Your participation in this research is entirely voluntary. It is your choice whether to participate or not. The choice that you make will have no bearing on your job or any work-related evaluations or reports. You may change your mind later and stop participating even if you agreed earlier.

Procedures

We are asking you to help us explore the mediating role of grit in the relationship between workplace environment and nurses turnover intention among nurses in major hospitals in Benguet Philippines. We are inviting you to take part in this research project by answering some questionnaires.

We will secure ethics clearance from your respective institution. After confirming the ethics clearance, a letter will be sent to the respective gatekeepers of your institution, Medical Center Chief through Chief Nurses, asking for permission to conduct the study. The letter addressed to the gatekeepers will also request the prospective respondents' institutional or personal email addresses. Politely, head nurses will be shown the endorsed letter by the Medical Center Chief to select nurses to participate in the study. Then when approval to float the questionnaire is secured, the researchers will float the questionnaire.

If you accept, you will be asked to fill up a questionnaire which will be provided to you through your emails. Responses will be automatically collated by the google forms and rest assured that the recorded information is confidential. Your name will not be included in the forms as we will use codes such as numbers for each of your responses and only these codes can identify you. No one else will be able to have access to your survey but the researchers.

Duration

The research will take place over 10 months in total from August, 2021-May, 2022. The consent may be taken virtually via google meet and google form to reduce human contact, or face to face if necessary, which will last for 15-30 minutes. After confirmation of participation, we will provide you with google form questionnaires regarding grit, workplace environment and turnover intention that can be filled in 15-20 minutes. Follow up after 3 days in email and follow up after a week through SMS will be done.

Risks

There are minimal risks associated with participation in the study. The primary modality of contact will be online. Also, we are asking you to share with us some personal and confidential information, and you may feel uncomfortable while answering about some of the topics. We will take at least 30 minutes of your time. If you feel the need to refuse to participate, you do not have to answer any question or take part in the questionnaire. That is fine. You do not have to give us any reason for refusing to take part in the interview.

In cases of face to face, we will make sure to follow the IATF protocols such as wearing K95 face masks and face shields and observing social distance to minimize the spread of the COVID-19 virus and to make sure all possible contacts will be safe for both you and us.

Benefits

The proposed study can help illuminate the reasons for the fast turnover of nurses in the Philippines. Specifically, in nursing practice, the proposed research will provide a model in limiting the pace of job turnover and further incrementing the employee's work commitment. This study will also help improve, monitor, and remove the practices currently applied and removed by evidence-based practices. In nursing education, the results of this study may be used to increase nurses' positive occupational perceptions through education, training, and lectures to retain future nurses in the healthcare setting. Nursing professors may also utilize various methods or interventions that increase the students' resilience, self-efficacy, motivation, and grit that could impact their nursing practice and retention in the nursing profession in the future.

The proposed study will be beneficial to increase the nurses' psychological capital and improve workplace environment conditions to reduce turnover and enable the nurses to perform better care for their patients. The study will contribute to developing organizational, institutional, and governmental solutions and strategies to reduce turnover rates for hospital nurses. This study will benefit you as it may become a means for hospital institutions to acknowledge the needs for interventions in addressing turnover intention of nurses. Furthermore, your participation is likely to help us find out more about the mediating role of grit in the relationship between workplace environment and turnover intention.

Reimbursements

You will not be provided any incentive to take part in the research study.

Confidentiality

The information that we will collect from this research project will be kept private and confidential. No personal identifiable information will be collected such as, but is not limited to: name, address, phone number, government issued numbers and photographs. The researchers will not share any information or linkages with anyone besides the research team and the panel. The researchers will comply with the Data Privacy Act of 2012 and the Privacy Guidelines of Saint Louis University.

To ensure confidentiality of data, the responses collected via google forms will be exported to a security enhanced and data encrypted online cloud, the Tresorit Software and will only be accessible to the principal investigator and the researchers. In cases where paper questionnaires are utilized, the responses will be kept in a padlocked cabinet in the university and will only be accessible to the principal investigator and the co-investigators.

Sharing the Results

Nothing that you tell us today will be shared with anybody outside the research team, and nothing will be attributed to you by name. The information that we get from this research will be shared with you and your institution before it is made widely available to the public. Each participant will receive a summary of the results. Summary of results and findings will be shared in an understandable and concise manner during the staff meeting, which would take fifteen

minutes. Moreover, the entire paper is planned to be submitted in a SCOPUS-indexed journal. The researchers also plan to present the findings in conferences both in the country and abroad.

Right to Refuse or Withdraw

You do not have to take part in this research if you do not wish to do so, and choosing to participate will not affect your job or job-related evaluations in any way. You may stop participating in answering the questionnaires at any time that you wish without your job being affected.

Who to Contact

If you have any questions, you can ask them during the google meet or face to face meeting when informed consent is being taken. If you wish to ask questions later, you may contact any of the following: YYYY

This proposal has been reviewed by the Research Ethics Committee of Saint Louis University, which is supporting the study. If you wish to find more about the REC, you may contact the chairperson and contact person, PPPP, Telephone number: (074) 442 2842, Email: recchair@slu.edu.ph

Part II: Certificate of Consent

I have been invited to participate in the study “The mediating role of grit in the relationship between workplace environment and turnover intention of nurses.” I have discreetly read the information sheet, and I also had the opportunity to ask questions and clarify any queries about the said study which have been answered to my satisfaction. I am aware that any information that I will share will ONLY be accessed by the researchers and the panelists as the information will solely be used for research purposes.

Print Name of Participant _____

Signature of Participant _____

Date _____

Day/month/year

Statement by the researcher/person taking consent

I have accurately read out the information sheet to the potential participant, and to the best of my ability made sure that the participant understands that the following will be done:

1. Informed Consent will be taken
2. Questionnaires will be answered

I confirm that the participant was given an opportunity to ask questions about the study, and all the questions asked by the participant have been answered correctly and to the best of my ability. I confirm that the individual has not been coerced into giving consent, and the consent has been given freely and voluntarily

A copy of this ICF has been provided to the participant.

Print Name of Researcher/person taking the consent_____

Signature of Researcher /person taking the consent_____

Date _____
Day/month/year

C. Project Gantt Chart

**Project title: MEDIATING ROLE OF GRIT IN THE RELATIONSHIP BETWEEN
WORKPLACE ENVIRONMENT AND TURNOVER INTENTION AMONG NURSES**

A. ACTIVITY PLAN GANTT CHART

Activity	Month									
	Aug	Sept	Oct	Nov	Dec	Jan	Feb	Mar	Apr	May
A. Proposal preparation										
1. Project brainstorming										
2. Review of literature										
3. Identifying the research problem and objectives										
4. Writing Introduction										
5. Research design and methodology										
6. Tool finalizations (questionnaire, etc.)										
7. Validation and pre-test										
8. Finalizing research proposal										
B. Data gathering										
1. Serving survey questionnaires										
2. Tabulating data										
C. Processing of data										
1. Descriptive analysis of profile										
2. Testing of the statistical model										
3. Data validation										
D. Final report/output preparation										
1. Writing										
2. Editing										
E. Paper presentation/ Publication										

D. CV of the Principal Investigator XXXX

Name:XXXX **Date:** December 16, 2024 **Email:** mariaclara.reyes@email.com **Mobile No.:** +63 912 345 6789

1. Educational Qualifications and Training

1.1 Relevant Degree(s):

- PhD in Public Health, University of the Philippines Manila, 2020
- Master of Science in Epidemiology, University of the Philippines Manila, 2015
- Bachelor of Science in Nursing, Saint Louis University, 2012

1.2 Research Methods Training:

- "Advanced Quantitative Research Methods" Workshop, Philippine Statistical Research and Training Institute, 2018
- "Qualitative Research in Health" Training Course, World Health Organization, 2016
- Certified in Research Ethics, National Institutes of Health (Online Course), 2022

1.3 Discipline-Specific Knowledge:

- Extensive knowledge of ethical guidelines and regulations for health research in the Philippines, including the National Ethical Guidelines for Health Research.
- Familiar with data privacy laws and regulations, such as the Data Privacy Act of 2012.

2. Experience

2.1 Prior Research Experience:

- Principal Investigator: "Impact of Community Health Worker Interventions on Maternal and Child Health in Rural Communities," funded by the Department of Health, 2021-2024.
- Co-Investigator: "Prevalence and Risk Factors of Non-Communicable Diseases among Urban Poor in Metro Manila," published in the *Philippine Journal of Public Health*, 2019.
- Research Assistant: "Assessment of Healthcare Access in Conflict-Affected Areas in Mindanao," 2013-2014.

2.2 Experience with Target Population:

- Extensive experience working with mothers and children in rural communities through various health programs and research projects.
- Trained in culturally sensitive approaches to research and community engagement.

2.3 Supervisory Experience:

- Supervised and mentored junior researchers and research assistants in data collection, analysis, and report writing.
- Provided guidance on ethical research practices and data management.

3. Knowledge of Ethical Principles

3.1 Understanding of core ethical principles:

- Demonstrates a strong understanding of informed consent, confidentiality, beneficence, non-maleficence, and justice in research.
- Committed to protecting the rights and welfare of research participants.

3.2 Awareness of relevant regulations:

- Familiar with national and international ethical guidelines, including the Belmont Report, the Declaration of Helsinki, and the CIOMS International Ethical Guidelines for Health-related Research Involving Humans.